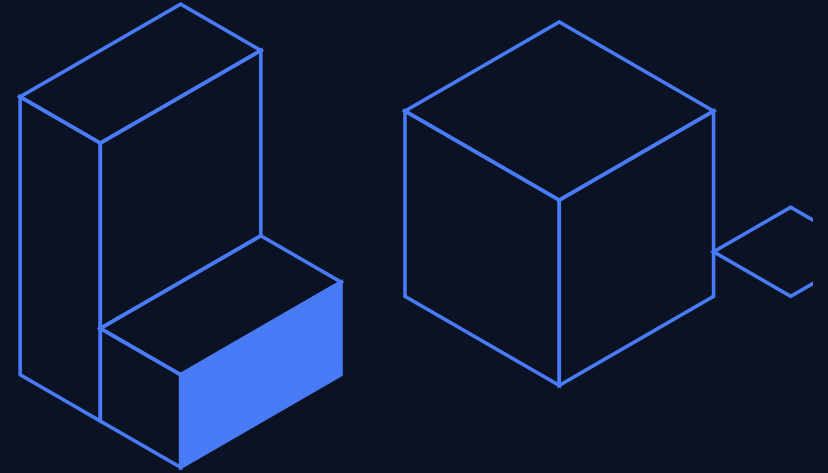




Your patient data is not trapped.

An introduction to the two firms in the room, and the one thing we checked before we met you.



Prepared by **Toggle Solutions** with Milkyway Marketing · 2 August 2026

[Your Digital Growth Partner]

[toggle.solutions]

Who you are working with

Two firms, one job. Toggle leads the engagement and owns everything technical. Milkyway is our delivery partner on paid media and messaging, and they are the ones who brought us your case.

LEAD PARTNER

Toggle Solutions

A Malaysian growth firm that diagnoses the whole revenue system before spending on any channel. Senior operators only, with no junior layer between you and the work. On your account we own the data, the tracking, and the connection between your clinic software and your marketing.

32,000+

leads · UNITAR

-47%

cost per lead

11.5x

ROI · Al Hidayah

DELIVERY PARTNER

Milkyway Marketing

An AI-led growth agency running paid media and automated messaging for businesses across Malaysia and the region. On your account they run the ad campaigns, the chatbot, and the creative production.

SLEEK

iMOTORBIKE

SPEEDHOME

PARCELDAILY

ANGLO HEALTH

PINNACLE

How we work

Diagnosis before prescription, every time. We would rather spend two weeks proving what is actually wrong than sell you twelve months of the wrong fix.

01

Diagnose

Find what is really limiting growth before recommending anything.

02

Prove it

Test the assumption cheaply and show you the result in writing.

03

Build

Ads, messaging and the data connection between them.

04

Feed it back

Send real outcomes to the ad platforms so they learn what a good patient looks like.

And the honest part: if the diagnosis says your problem is not marketing, we say so. We have walked away from a retainer for exactly that reason and said so publicly. You will hear the same directness from us on your software question today.

WHAT WE CHECKED BEFORE TODAY

Your data is not trapped

We were told getting data out of Kreloses is the thing blocking you. So we went and read what Kreloses says about it themselves.

"Your data always belongs to you. You can export your complete database whenever you want, no restrictions and no fees."

Kreloses pricing page, checked 29 July 2026

That is on their public pricing page and you can check it in five minutes. Our read is that the export does work, and that whoever tried it received raw files with internal codes and no labels, which is unusable to anybody running a clinic. **That is a formatting problem, not a lock-in problem**, and it changes what your options actually are.

What we still need to check

Everything we know so far came to us second hand, so we are not going to present it back to you as fact. Three questions decide the whole shape of this, and only you can answer them.

What exactly fails on uploads?

Is it file size, the number of files per patient, total storage, or file types it refuses? For a veneers practice your photos and scans are the case record, so this is the question that decides whether you should change systems at all.

What does your Kreloses agreement allow?

Before we automate anything against your account we want to read your subscription terms and confirm what automated access they permit. We are not going to put your live clinic system at risk to save you a manual step.

Where do enquiries land today?

Who answers them, how fast, and what happens to the ones that do not book. This tells us whether your problem is reach, response, or follow-up, and those need three different fixes.

What we need from you today

Give us these and we can come back with a costed recommendation rather than a brochure. If the answer turns out to be simple, we will tell you that and skip the second meeting.

To understand the practice

- 01** Roughly how many enquiries come in a month, and how many turn into consultations
- 02** What an average veneers case is worth to you
- 03** Who handles enquiries now, and on which channels
- 04** What you mean by a chatbot: WhatsApp, Instagram, or your website

To scope the work

- 05** A budget range, so we recommend something you would actually buy
- 06** Your Kreloses subscription agreement, and which plan you are on
- 07** Fifteen minutes screen sharing Kreloses with whoever uses it daily
- 08** Who signs off marketing, since your dentist has to approve creative

What you get back: a written answer on whether to keep or change your software, and a costed plan for the marketing. Nothing in this deck commits you to either.

What you told us

Six gaps in Kreloses, and you named three of them as the ones that hurt today. We went and checked all six before writing anything.

1. Getting data out is hard

Makes switching to anything else feel risky.

LIVE PAIN

2. No API

Nothing can be automated with other tools.

LIVE PAIN

3. Limited uploads per patient

Including your photos and scans.

LIVE PAIN

4. No patient app

Nothing patients can carry with them.

5. No broadcast campaigns

No way to message the base at once.

6. No loyalty program

No structured way to reward returning patients.

What you want to build: ads that bring in the right patients, and a chatbot that answers them and books consultations. Your worry was that no chatbot can connect to Kreloses. That worry is the reason this meeting exists, and it turns out to be smaller than it looks.

THE METHOD

How we get your data out

Two routes, and neither one needs Kreloses to build us anything or to give us permission. Both run on access you already own.

ROUTE 1

The export they already offer

You request your complete database, which their pricing page says costs nothing and carries no restrictions. It arrives as raw files with internal codes and no labels, and we turn that into something usable. Slower to refresh, and it is the sanctioned route, in writing.

ROUTE 2

Your own login, on a schedule

Every time your staff open a screen, Kreloses loads that information as structured data behind the page. Using your own account we can collect the same information on a schedule, which gives us appointments and recall dates without waiting on an export.

We confirm which route works on your live account in week one, before anything is built. We also read your subscription agreement first, because we are not going to automate against your clinic system in a way its own terms do not allow. If Route 2 is off limits, Route 1 still carries most of what the funnel needs.

What that unlocks

Once the data moves freely, the software question stops being a trap and becomes a choice. All three of these open up at once.

Move, without the risk

If another system suits a veneers practice better, you can go, and your patient history goes with you.

Stay, without the trap

Kreloses covers your clinical records well. Staying becomes a decision you made rather than one made for you.

Or build the missing pieces

Keep Kreloses for records and add only what it lacks, sitting alongside it rather than replacing it.

Our honest recommendation: **do not change systems yet**. Changing the record system of a practice running year-long treatment plans is real clinical risk, and right now you would be taking it for reasons that turn out not to hold. One reason might still justify it, and we come to that on slide 8.

FINDING 2 OF 2

The chatbot was never blocked by Kreloses

Here is every step of the funnel you described, and what each one needs from your patient system. Somebody who clicks an ad is not a patient yet, so there is nothing to look up.

1	Ads on Google and Meta	NEEDS NOTHING
2	Landing page and enquiry capture	NEEDS NOTHING
3	Chatbot answers and qualifies Veneer options, price ranges, how long treatment takes	NEEDS NOTHING
4	Follow up with people who did not book	NEEDS NOTHING
5	Booking the consultation Solved by the bot booking into our calendar, front desk copies it across	WRITES IN
6	Appointment reminders Only needed for walk-ins and phone bookings the funnel did not make	PARTLY
7	Recall and bringing past patients back Solved by a scheduled export feeding the campaign list	READS OUT

4 of 7 steps never touch Kreloses at all

2 of 7 need it, and both have a workaround

0 of it requires you to change software

The system we would run

Toggle designs and owns this system end to end, and Milkyway runs the media and messaging inside it. Ads without fast replies means you pay for enquiries that go cold. Fast replies without good targeting means you answer the wrong people quickly.

PART 1 · MEDIA · MILKYWAY

Reaching people already looking

Campaigns across Google Search, Display, YouTube, Facebook and Instagram, aimed at people actively researching veneers in your area. Continuous creative and audience testing, with budget shifted toward whatever is producing consultations.

PART 2 · MESSAGING · MILKYWAY

Replying in under a minute, around the clock

An assistant on WhatsApp, Instagram DM and your website that answers the common veneer questions, qualifies the enquiry, books the consultation, and chases anyone who goes quiet. Built on an official Meta WhatsApp business platform.

PART 3 · THE DATA LAYER · TOGGLE

The connection to your clinic

Getting real appointment and recall information out of Kreloses so the first two parts run on what is actually happening in your practice, instead of guessing. This is the piece that has been missing, and it is the next slide.

Nobody buys veneers in a chat window

This is why ad money leaks on a practice like yours. Google and Meta only learn from what they can see, and right now they cannot see the part that matters.

01

Someone enquires

Meta sees this and records a win.

02

They book a consult

Meta sees nothing.

03

They attend

Meta sees nothing.

04

They accept a case

Meta sees nothing. This is the only step worth money.

What that costs you

Because the only signal Meta gets is the enquiry, it goes and finds you more enquiries. Cheap ones. It learns nothing about which ad produced a patient who sat in your chair and said yes to a full set of veneers, so it cannot go find more of those.

What we do about it

We read attendance and case value out of Kreloses and feed them back to Google and Meta as the real result. The platforms stop chasing cheap enquiries and start chasing accepted cases. On a practice where one case is worth many times a routine visit, that changes what the same budget buys.

The one thing this does not fix

Of the six gaps you listed, this plan handles five without you changing any software. We are not going to pretend it handles the sixth.

Gap 2 · No API

Handled. We pull what the funnel needs with your own access.

Gap 1 · Data export

Handled. The export exists and we make it usable.

Gap 5 · Broadcasts

Handled outside Kreloses, on the messaging platform.

Gap 4 · No patient app

Handled better. Patients already have WhatsApp installed.

Gap 6 · Loyalty

Handled outside Kreloses, once the funnel is live.

Gap 3 · Photos and scans

Not handled. Nothing we build outside Kreloses can raise a storage limit.

Your before and after photos, your scans and your smile mockups are the clinical record and the evidence of a case that runs over a year. If Kreloses caps what you can attach per patient, that is the one reason worth changing systems over, and it has nothing to do with the API. **Tell us exactly what fails there**, whether it is file size, the number of files, total storage or file types, and we will tell you honestly whether to move.

Two rules we design around from day one

Neither one blocks the work. Both cost far less to build in now than to unpick after a complaint.

Malaysian Dental Council advertising rules

Dental advertising is restricted, and elective aesthetic work like veneers is the category watched hardest. Patient testimonials that read as endorsement, superlative claims, guaranteed outcomes and patient solicitation are all limited.

Published research on Malaysian dental clinic social accounts found around **15 percent of posts non-compliant**, and more than half of the accounts studied carried at least one. Every piece of creative goes to your dentist for sign-off before it runs.

PDPA and your patients' data

Malaysian law treats health information as sensitive personal data, which needs explicit consent to process, and patient data cannot be used to market services without it. The 2024 amendments added mandatory breach reporting and higher penalties.

In practice: the chatbot asks for consent before it collects anything, and we only message your existing patients if they agreed to marketing contact.

Consenting to treatment does not cover promotions.

We also design the ad measurement so that attendance is reported back against an advertising click reference rather than against a named patient, which keeps identifiable health information out of the ad platforms entirely.

How we would start

Phase 1 is deliberately small and cheap, because it answers the questions that decide everything after it. Timings are indicative and firm up once Phase 1 is done.

PHASE	ROUGHLY	WHAT HAPPENS	WHO LEADS
Phase 1	Weeks 1 to 2	Prove the data comes out We request your full Kreloses export and inspect it, confirm what the upload limit actually is, and read your Kreloses subscription agreement so we know what automated access it permits. You get a written answer on whether to change systems.	Toggle
Phase 2	Weeks 2 to 6	Ads and the assistant go live Campaign build, landing page, and the chatbot on WhatsApp and Instagram with a consent step. Creative goes to your dentist for MDC sign-off before anything runs. Bookings land in a shared calendar your front desk copies into Kreloses.	Milkyway
Phase 3	Weeks 6 to 10	Close the loop Attendance and case values start flowing back to Google and Meta, so the platforms begin optimizing toward accepted cases. Recall and reactivation campaigns switch on for patients who consented.	Toggle

Being straight about timing: the ads can be live inside a few weeks. The full loop back from your clinic to the ad platforms takes longer, because it depends on what Phase 1 finds. We would rather tell you that now than at week two.

Who does what

Toggle leads the engagement and carries the result. Milkyway runs the media and messaging inside it. One line of accountability, and nothing falls between the two of us.

TOGGLE LEADS

Toggle Solutions

Accountable for the engagement end to end. Getting data out of Kreloses, the connection between your clinic and the funnel, feeding results back to the ad platforms, and the compliance design across MDC and PDPA.

32,000+

leads · UNITAR

-47%

cost per lead

11.5x

ROI · Al Hidayah

MILKYWAY DELIVERS

Milkyway Marketing

The ad campaigns across Google and Meta, the chatbot and messaging platform, creative production, and the monthly campaign reporting.

SLEEK

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SPEEDHOME

PARCELDAILY

ANGLO HEALTH

PINNACLE

One thing we will not do: let any automated system write into your clinical calendar. Bookings arrive in a shared calendar and your front desk moves them across. It takes seconds, and it means no software bug can ever wipe a real patient's appointment.



Let's prove the data comes out first.

Phase 1 answers the question you walked in with, and it does it before you commit to any campaign spend or any change of software.



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